

Projected Health Impact of Achieving WHO Targets for Cardiovascular Risk-Factor Control

Work in Progress

WHO · University of Washington

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Study Overview

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Study Overview

Aim 1: Research Questions

Central Question

What is the projected **burden-of-disease impact** of achieving the WHO cardiovascular risk-factor control targets by **2030**?

Sub-questions driving the analysis:

1. How many **cardiovascular events** could be averted?
2. How many **CVD deaths** could be prevented by 2030 and beyond?
3. How would **life-years, YLLs, YLDs, and DALYs** change under joint achievement of:
 - 150M additional people with controlled hypertension
 - $\geq 50\%$ lipid-lowering therapy coverage
 - 80% BP control among people with diabetes
 - **80% validated BP device availability**

Scope

190 countries · Ages 20–95 · 2026–2050 ·
Multiple CVD causes · WHO Target Scenarios

Intervention start: 2026

Model calibrated to 2019 baseline. First year of target scale-up effects entering the projection is **2026**. Aim 1: Modelling Study

Study Design at a Glance

Component	Detail
Model type	Discrete-time multi-state model (Well → Sick → Dead)
Calibration data	GBD 2023: incidence, case fatality, prevalence; all-cause & CVD-specific
Population data	UNWPP 2024 single-year age/sex projections
Intervention start	2026 — first year intervention effects enter the projection
Scale-up period	Linear scale-up 2026 → 2030; rates held constant 2030–2050
Comparison	Business-as-usual: 2024 control rates, no additional scale-up
Outcome	CVD deaths averted vs. BAU; age-standardised mortality rate (ASMR)
Time horizon	2026–2050 (25-year horizon); model calibrated from 2019

Study Context

Objective

- Quantify CVD deaths avertable by achieving WHO 2030 cardiovascular targets
- Coverage: **190 countries**, 2025–2050
- Four causes: IHD, ischemic stroke, hemorrhagic stroke, hypertensive HD

Model

- Discrete-time state transition (Well → Sick → Dead)
- Calibrated to GBD 2023; projected with UNWPP 2024 populations
- 8 blood-pressure bins
- Baseline treatment effects explicitly de-duplicated

Research Question

What are the projected deaths averted from achieving WHO hypertension control and lipid-lowering therapy targets across **190 countries**, 2026–2050?

Pending 

Confirm primary metric: deaths averted vs. DALYs vs. LYG

Data Sources

Data Sources and Key Variables

Source	Variables extracted	Role in model
GBD 2023 (IHME)	IR, CF, prevalence, background mortality (age/sex/country)	Baseline transition rates; initial states
GBD 2019 (IHME)	Relative risks per 10 mmHg SBP increase (IHD, stroke, HHD, by age)	Anchor BP-bin incidence; normalisation factor α
Ettehad et al. 2016	Trial effect sizes per BP category; diabetes-stratified	Treatment effect sizes by BP bin $\times$ cause
UNWPP 2024 (UN)	Population projections 2019–2050 (single-year age, sex, country)	Cohort sizes; rate denominators
NCD-RisC	Baseline HTN control rates (C_0); BP distribution by country	C_0 ; $P(\text{BP}_{\text{cat}})$
Country-specific	HTN control targets 2030 (Progress / Ambitious / Aspirational)	C_{target} per scenario

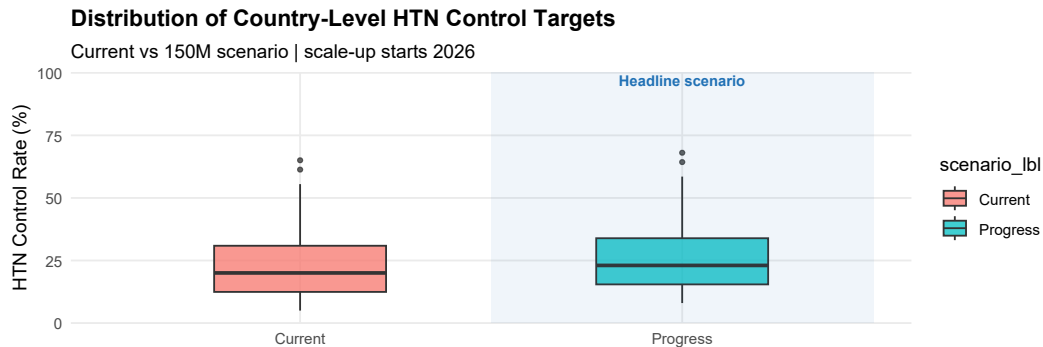
Scenarios

Four WHO Priority Targets

#	WHO Target	Modelled Scenario
1	150M more with HTN controlled by 2030	HTN Control (General): $\approx$ 100M more people with BP control by 2030
2	$\geq$ 50% eligible adults on lipid-lowering therapy by 2030	Improved Statin Uptake: 50% coverage by 2030
3	80% BP control among diagnosed diabetics by 2030	HTN Control (Diabetes): 80% BP control in diabetics by 2030 ($\approx$ 50M more people with BP control)
4	80% availability of validated BP devices	Operationally embedded in HTN scenarios (150M Target [1+3])

Combination rule: $\varepsilon_{IR, \text{total}} = \varepsilon_{IR, \text{BP}} \times \varepsilon_{IR, \text{statin}}$

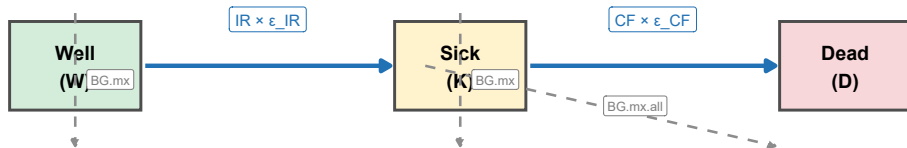
HTN Control Targets by 2030 — Country Distribution



Mathematical Model

Model Architecture: Three Health States

Multi-State Transition Model | Annual time step | Ages 20-95 | 4 CVD causes



Key: ε_{IR} , ε_{CF} = multiplicative effect ratios from the intervention module · BG.mx = background mortality · COVID-19 excess mortality applied 2020–2021 only

Modeling Assumptions

Model structure

- Discrete-time annual step (2019–2050); ages 20–95
- Four causes modeled independently
- COVID-19 excess mortality applied 2020–2021 only; no long-term effects

Intervention mechanics

- Linear scale-up 2026 → 2030; held constant 2030–2050
- Only incremental coverage above C_0 (or $U_{0,\ell}$) generates new effects (no double-counting)
- Full adherence assumed for HTN control, statins adherence adjusted down for primary and secondary prevention.
- Statin effects restricted to ages ≥ 40 and IHD & ischemic stroke only

Blood pressure & effect sizes

- Static log-normal BP distribution: no secular trend, no endogenous updating as coverage rises
- Treatment eligibility: ≥ 140 mmHg (5 hypertensive bins)
- GBD RRs anchor bin-level baseline incidence
- Diabetes subgroup: base control rates assumed 15 percentage points higher than general HTN
- Diabetes subgroup: multiplicative reduction blended with general HTN effects using diabetes prevalence and high blood pressure prevalence as weights

Combination of interventions

- Effects combine **multiplicatively** across all interventions
- BP-related effects combined first, then with statins
- No saturation, synergy, or interaction terms beyond multiplicative combination

Burden of Disease: Methods

Three burden-of-disease metrics

- **YLL** (Years of Life Lost): deaths $\times$ remaining life expectancy at age of death
- **YLD** (Years Lived with Disability): prevalent CVD cases $\times$ disability weight
- **DALY** (Disability-Adjusted Life Years):
$$DALY = YLL + YLD$$

$$\text{DALYs averted} = DALY_{\text{BAU}} - DALY_{\text{intervention}}$$

Full derivation in Supplemental Mathematical Details.

Reference standards

Life table: UNWPP 2024, medium variant (country- and year-specific e_x)

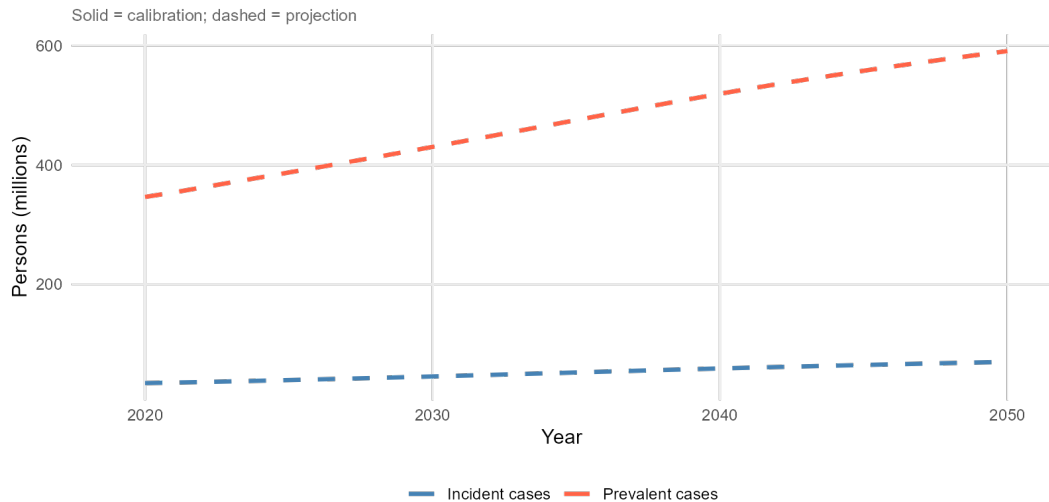
Disability weights: GBD 2023, derived as YLD/Prevalence per cause and location

Discounting: None (undiscounted)

Age weighting: None

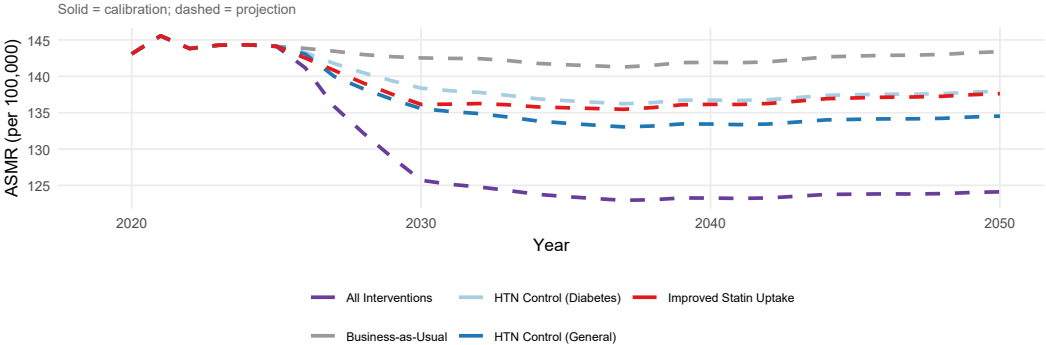
Results

Baseline Disease Trends (Business-as-Usual)



Incident and prevalent CVD cases 2019–2050; solid = calibration, dashed = projection.

Age-Standardised CVD Mortality Rate



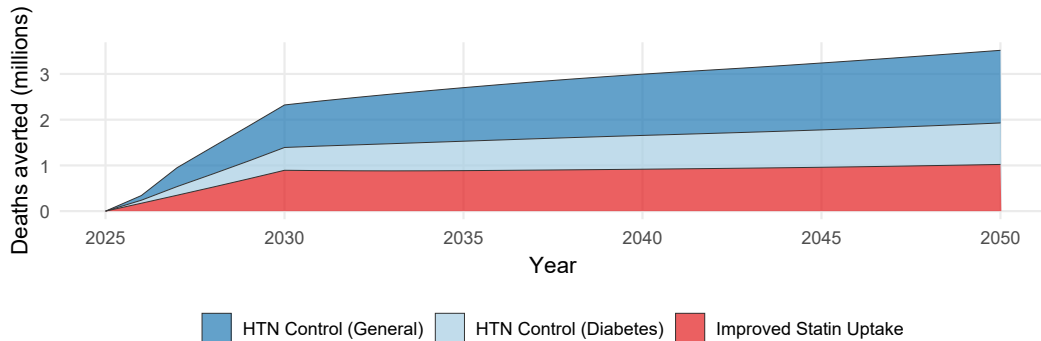
Summary: Cumulative Deaths Averted, 2026–2050

Scenario	Cumulative (M)	Avg./year (M)	% of BAU deaths
HTN Control (General)	29.4 M	1.2 M	4.2%
HTN Control (Diabetes)	16.2 M	0.6 M	2.3%
Improved Statin Uptake	21.3 M	0.9 M	3.1%
All Interventions	65.6 M	2.6 M	9.4%

Headline

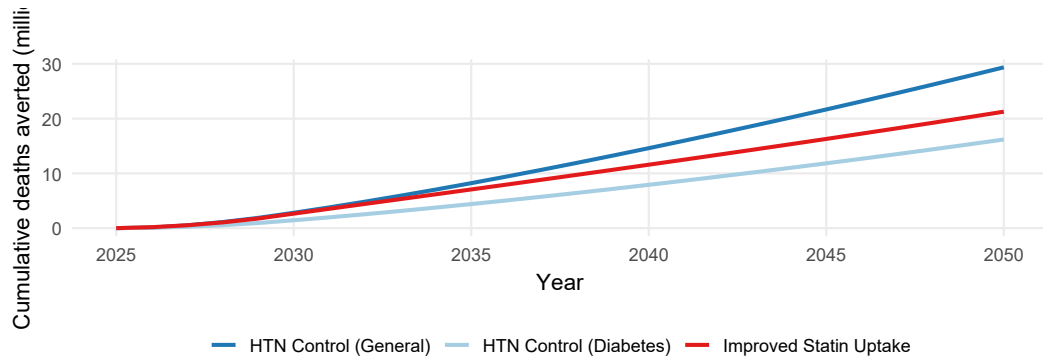
All Interventions: 65.6 million deaths averted globally, 2026–2050 ($\approx$ 2.6 M/year).

Annual Deaths Averted Over Time



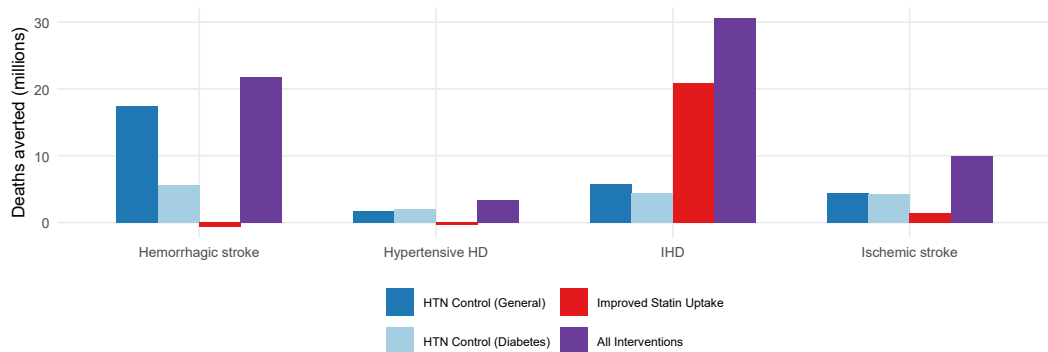
Cumulative deaths averted per intervention relative to business-as-usual.

Cumulative Deaths Averted Over Time

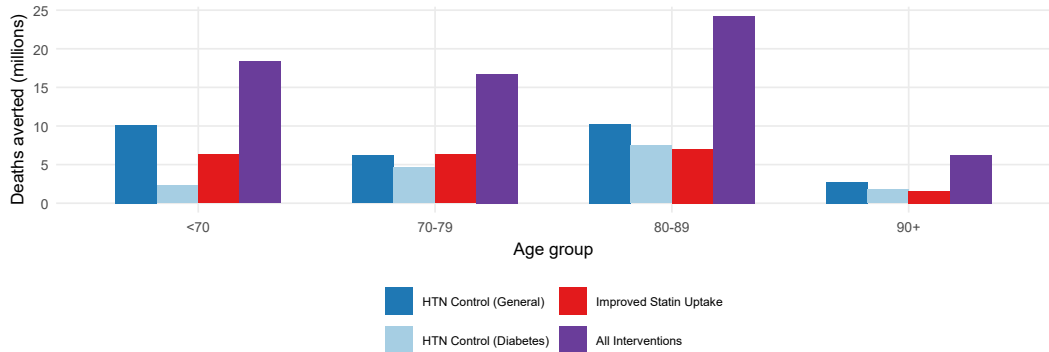


Cumulative deaths averted per intervention relative to business-as-usual.

Deaths Averted by Cause of Death



Deaths Averted by Age Group



Burden of Disease Averted: YLLs, YLDs, and DALYs

- DALYs averted: 787.0 M
- YLLs averted: 769.3 M
- YLDs averted: 17.7 M

Intervention	Deaths averted	DALYs averted	YLDs averted	YLLs averted
HTN Control (General)	29.4 M	376.2 M	14.0 M	390.2 M
HTN Control (Diabetes)	16.2 M	144.5 M	5.4 M	149.8 M
Improved Statin Uptake	21.3 M	264.8 M	-1.7 M	263.1 M
All Interventions	65.6 M	769.3 M	17.7 M	787.0 M

Headline

The combined WHO target package generates large health gains, with most benefits coming through avoided premature CVD deaths rather than morbidity reduction alone.

Regional Summary

Region	BAU deaths (M)	Averted (M)	% averted
Western Pacific	227.3	21.7	9.5%
South-East Asia	175.8	17.2	9.8%
Europe	117.2	9.8	8.3%
Africa	48.7	5.9	12.2%
Eastern Mediterranean	64.3	5.8	9.1%
Americas	62.8	5.2	8.2%

Observation

Absolute gains largest in high-burden regions; proportional gains reflect current coverage gaps.

Discussion

Interpretation

Key findings

- **All Interventions** yields largest gains: HTN control + statins are complementary
- **IHD** and **ischemic stroke** drive the majority of avertable deaths
- Gains **accelerate after 2030** as coverage compounds
- **Older adults (70+)** benefit most in absolute terms
- **Top 15 countries** account for $>75\%$ of avertable burden

Policy Implication

Combined WHO Targets 1 + 2 are synergistic — neither alone captures the full benefit.

Uncertainty

Sensitivity analysis to diabetes targeting assumptions and secular trends pending.

Limitations

Model structure & assumptions

- Stable transition rates assumed; no dynamic feedback between states
- Static BP distribution (log-normal): no secular trend and no endogenous updating as coverage increases
- No heterogeneity in adherence: newly HTN covered individuals assumed fully adherent
- COVID-19 excess mortality only modelled for 2020–2021; long-term effects excluded

Data & parameters

- RCT-based effect sizes (e.g., Ettehad ρ_{CF}): real-world effectiveness at scale may be lower
- GBD 2023 uncertainty intervals not propagated
- Baseline C_0 from NCD-RisC; measurement error unquantified
- BP distributions rely on GBD modelling (not direct surveys in all countries)

Scenarios & implementation realism

- Linear scale-up assumed (2026–2030); real programmes may follow non-linear trajectories
- No explicit health-system capacity constraints modelled
- Interactions with statins, diabetes care, and **BP device availability** not included

Next Steps

Analysis priorities

1. Sensitivity analysis on effect sizes, trends and coverage
2. Add life-years gained (LYG) and DALYs as co-primary outcome
3. Stratify by income group (World Bank)
4. Conduct Aim 2: simulate alternative scale-up trajectories and implementation scenarios for hypertension control

Pending Decisions

- Journal target?

Summary

1. The gains are large and achievable

All-interventions: **65.6 million deaths averted** globally, 2026–2050.

2. HTN control + statins compound risk reduction

HTN control and statins have multiplicative risk reductions, producing larger combined effects.

3. Regional patterns reflect burden and coverage gaps

Absolute gains largest in high-burden regions; proportional gains reflect current coverage gaps.

Meeting WHO 2030 CVD targets is not just aspirational — it is quantifiably lifesaving.

Appendix

Notation: Core Model

Symbol	Description
<i>Core state-transition model</i>	
a, s, c, t, ℓ	Age, sex, cause, year, location
W, K, D	Well, Sick, Dead populations
$IR_{a,s,c}, CF_{a,s,c}$	Baseline incidence & case-fatality rates
$\varepsilon_{IR,j}, \varepsilon_{CF,j}$	Multiplicative effect ratios for intervention j
$BG.mx, BG.mx.all$	Cause-specific & all-cause background mortality
$covid.mx$	COVID-19 mortality (2020–2021 only)

Notation: HTN Control

Symbol	Description
<i>HTN control (general & diabetes subgroup)</i>	
$C_0, C_t, C_{\text{target}}$	Baseline, time- t , and target control rates
$\Delta C_{\text{target}}, \Delta C_{\text{increment}}$	Total & time-varying incremental coverage
C_{agg}^{Δ}	Population-weighted incremental coverage (hypertensive bins)
$P(\text{BP}_{\text{cat}})$	BP category population distribution (8 bins)
RR_{GBD}	GBD relative risk per 10 mmHg SBP increase
α_{inc}	Incidence normalization factor
$E_{\text{trial}}, E_{\text{DM}}, E_{\text{noDM}}$	Ettehad effect sizes (blended, DM-only, non-DM)
w_{DM}	Diabetes prevalence weight (general HTN: 0.10; DM subgroup: 1.0)
$\rho_{CF,c}$	CF reduction factor per unit incremental coverage
$T_{\text{start}}, T_{\text{end}}$	Scale-up start (2026) & end (2030)

Notation: Statins & Combined Effects

Symbol	Description
<i>Statin therapy</i>	
$U_{0,\ell}, U_{\text{target}}$	Baseline & target statin coverage
$\Delta U(t, \ell)$	Incremental statin coverage above $U_{0,\ell}$ at time t
$T_{\text{stat,start}}, T_{\text{stat,end}}$	Statin scale-up start (2026) & end years
RR_c^{IR}, RR_c^{CF}	Trial-based RRs for incidence & case fatality
$\delta_c^{IR}, \delta_c^{CF}$	Risk reductions = $1 - RR$
$AF_{c,\ell}$	Cause- & location-specific attributable fraction (incidence only)
ψ_{athero}	Atherosclerotic fraction of ischemic stroke (= 0.60)
$E_{c,\ell}^{IR}(t), E_{c,\ell}^{CF}(t)$	Incremental effect sizes for IR and CF

HTN Intervention Module: 9-Step Pipeline

	Action	Output
1	Baseline BP distribution $P(\text{BP}_{\text{cat}})$ without treatment ($\text{rx} = 0$)	$P(\text{BP}_{\text{cat}} \mid a, s, t)$
2	GBD relative risks per 10 mmHg SBP $\rightarrow$ bin-specific incidence IR_{bin}	IR_{bin}
3	Diabetes-weighted Ettehad trial effect sizes E_{trial} per BP bin $\times$ cause	E_{trial}
4	Linear coverage scale-up from C_0 to C_{target} , starting 2026	$C_t(t, \text{BP}_{\text{cat}})$
5	Coverage adjustment: $E_{\text{adj}} = E_{\text{trial}}(C_t - C_0) / (1 - E_{\text{trial}} C_0)$	$E_{\text{adj}}(t)$
6	New bin incidence: $IR_{\text{bin,new}} = IR_{\text{bin}} \times (1 - E_{\text{adj}})$	$IR_{\text{bin,new}}$
7	Aggregate: $IR_{\text{new}} = \sum IR_{\text{bin,new}} \times P(\text{BP}_{\text{cat}})$	IR_{new}
8	Incidence effect ratio: $\varepsilon_{IR} = IR_{\text{new}} / IR_{\text{original}}$	$\varepsilon_{IR} \rightarrow \text{model}$
9	Case-fatality effect ratio: $\varepsilon_{CF} = 1 - \rho_{CF} \times C_{\text{agg}}^{\Delta}$	$\varepsilon_{CF} \rightarrow \text{model}$

Blue row = intervention start (2026). Green rows = outputs passed to state-transition model.

HTN Control: Coverage Scale-Up and Key Equations

Incidence pathway

Step 2 — GBD RR anchors bin incidence:

$$IR_{\text{bin}} = \frac{RR_{\text{GBD}} \times IR}{\alpha}, \quad \alpha = \sum_{\text{cat}} P_{\text{cat}} \cdot RR_{\text{GBD}}$$

Step 5 — baseline-adjusted effect (no double-counting):

$$E_{\text{adj}}(t) = \frac{E_{\text{trial}}(C_t - C_0)}{1 - E_{\text{trial}} C_0}$$

Steps 7–8 — population-weighted effect ratio:

$$\varepsilon_{IR} = \frac{\sum IR_{\text{bin,new}} \cdot P_{\text{cat}}}{IR_{\text{original}}}$$

Case fatality pathway

Incremental coverage above baseline:

$$\Delta C_{\delta}(t) = \max(C_t - C_0, 0)$$

Aggregate across hypertensive bins (≥ 140 mmHg):

$$C_{\text{agg}}^{\Delta} = \frac{\sum_{\geq 140} \Delta C_{\delta} \cdot P_{\text{cat}}}{\sum_{\geq 140} P_{\text{cat}}}$$

CF effect ratio:

$$\varepsilon_{CF} = 1 - \rho_{CF} \times C_{\text{agg}}^{\Delta}$$

Cause	ρ_{CF}
IHD	0.24
Ischemic stroke	0.36
Hemorrhagic stroke	0.76
HHD	0.20

HTN Control (Diabetes Subgroup)

WHO Target: 80% BP control among diagnosed diabetics by 2030

Same 9-step pipeline as general HTN. Key distinction: diabetes-specific Ettehad effect sizes ($w_{\text{DM}} = 1$):

General HTN (DM-blended):

$$E_{\text{trial}} = (1 - w_{\text{DM}}) E_{\text{noDM}} + w_{\text{DM}} E_{\text{DM}}, \quad w_{\text{DM}} = 0.10$$

Diabetes subgroup (DM-only):

$$E_{\text{trial}}^{\text{DM}} = E_{\text{DM}}$$

Coverage scale-up to $C_{\text{target}}^{\text{DM}} = 0.80$:

$$C_t^{\text{DM}} = C_0^{\text{DM}} + \Delta C^{\text{DM}} \min \left[\frac{t - 2026}{4}, 1 \right]$$

Effect ratios (same structure as general HTN):

$$\varepsilon_{IR, \text{DM}}(a, s, c, t) = \frac{IR_{\text{new}}^{\text{DM}}}{IR_{\text{original}}}$$

$$\varepsilon_{CF, \text{DM}} = 1 - \rho_{CF, c} \times C_{\text{agg}}^{\Delta, \text{DM}}$$

Why a separate module?

People with diabetes have higher CVD risk at any BP level. DM-specific Ettehad effects are larger (especially for stroke) than blended effects at $w_{\text{DM}} = 0.10$.

When combined with general HTN

Effects multiply (no double-counting via incremental coverage formulation):

$$\varepsilon_{IR, \text{BP}, \text{total}} = \varepsilon_{IR, \text{gen}} \times \varepsilon_{IR, \text{DM}}$$

Statin Therapy: Coverage Scale-Up and Key Equations

Scope: IHD & ischemic stroke; ages ≥ 40 only

Incremental coverage above baseline $U_{0,\ell}$:

$$\Delta U(t, \ell) = \Delta U_{\text{target}, \ell} \min \left[\frac{t - T_{\text{stat}, \text{start}} + 1}{T_{\text{stat}, \text{end}} - T_{\text{stat}, \text{start}} + 1}, 1 \right]$$

where $\Delta U_{\text{target}, \ell} = \max(U_{\text{target}} - U_{0,\ell}, 0)$

Trial-based RRs (per ≈ 1 mmol/L LDL-C reduction):

Cause	RR^{IR}	RR^{CF}
IHD	0.74	0.80
Ischemic stroke	0.80	0.96

Incidence (primary prevention, AF-scaled):

$$E_{c,\ell}^{IR}(t) = \frac{AF_{c,\ell} \delta_c^{IR} \Delta U(t, \ell)}{1 - \delta_c^{IR} U_{0,\ell}}$$

Case fatality (secondary prevention; no AF; atherosclerotic restriction for stroke):

$$E_{c,\ell}^{CF}(t) = \begin{cases} \frac{\delta_{\text{ihd}}^{CF} \Delta U(t, \ell)}{1 - \delta_{\text{ihd}}^{CF} U_{0,\ell}}, & c = \text{ihd} \\ \frac{\psi_{\text{athero}} \delta_{\text{ist}}^{CF} \Delta U(t, \ell)}{1 - \delta_{\text{ist}}^{CF} U_{0,\ell}}, & c = \text{istroke} \end{cases}$$

$\delta_c^{IR} = 1 - RR_c^{IR}$; $\psi_{\text{athero}} = 0.60$
(atherosclerotic fraction)

Effect ratios passed to model:

$$\varepsilon_{IR, \text{stat}} = \begin{cases} 1 - E_{c,\ell}^{IR}(t), & a \geq 40, c \in \{\text{ihd}, \text{istroke}\} \\ 1, & \text{otherwise} \end{cases}$$

$$\varepsilon_{CF, \text{stat}} = \begin{cases} 1 - E_{c,\ell}^{CF}(t), & a \geq 40, c \in \{\text{ihd}, \text{istroke}\} \\ 1, & \text{otherwise} \end{cases}$$

References I

- **Ettehad D, et al.** Blood pressure lowering for prevention of CVD and death. *Lancet*. 2016;387:957–967.
- **GBD 2023 Collaborators** (2024). Global mortality for 371 causes. *Lancet*.
- **Kontis V et al.** (2019). Contributions of risk factor trends to cardiometabolic mortality decline. *Nature Medicine*, 25, 579–584.
- **Mills KT, et al.** Global disparities of hypertension prevalence and control. *Circulation*. 2016;134:441–450.
- **Murray CJL et al.** (2020). Global burden of 87 risk factors: GBD 2019. *Lancet*, 396, 1223–1249.
- **NCD Countdown 2030 Collaborators.** Pathways to achieving SDG target 3.4. *Lancet*. 2022;399:1226–1249.
- **Pickersgill et al.** (2022). Modeling global 80-80-80 blood pressure targets and cardiovascular outcomes. In: *Nature Medicine*
- **UNWPP** (2024). World Population Prospects 2024. United Nations.
- **Watkins, D et al.** (2025). Global Impact of Single-Pill Combination Therapies on Cardiovascular Mortality and Events, 2023-2050: A Modeling Study. In: *JACC*.
- **WHO** (2023). Global Hearts Initiative: HEARTS Technical Package. Geneva.
- **WHO.** Global NCD Political Declaration. Geneva: WHO; 2025.

Thank You

Questions & Discussion

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